

HARBOR ORTHOPEDIC GROUP
New Patient Consultation Note

Patient: Okonkwo, Adaeze

DOB: 1979-02-14 MRN: HOG-2024-3318

Date of Service: 01/31/2024

Rendering Provider: Daniel Park, MD (Orthopedic Surgery, NPI 1356679920)

Encounter Type: New patient consultation, level 4 (CPT 99204, mod 25)

CHIEF COMPLAINT

Right knee pain x 10 weeks, refractory to conservative care.

HPI

44-year-old previously active female (recreational soccer, 3x/week) presents with insidious onset right medial-sided knee pain that began after a misstep on uneven ground in mid-November 2023. Pain is 6/10 at baseline, 8/10 with stair descent and prolonged sitting >30 min (theater sign positive). Reports occasional catching and one episode of give-way when stepping off a curb. No frank locking. Denies constitutional symptoms.

PRIOR CONSERVATIVE TREATMENT (failed)

- 11/20/2023 – 12/15/2023: NSAIDs (naproxen 500 mg BID x 4 weeks) and activity modification – partial relief, returned to baseline when resumed activity.
- 12/18/2023 – 01/19/2024: Formal physical therapy x 8 sessions (Harbor Rehab) focused on quadriceps strengthening and patellofemoral mechanics. Discharge note (Cano, PT, DPT) documents persistent medial-sided pain and effusion despite full home exercise compliance.

EXAM (right knee)

- Range of motion: 0-125° (left 0-135°)
- Small effusion present; ballottement positive
- McMurray maneuver: positive medial click with extension under valgus
- Joint line tenderness: medial > lateral
- Ligamentous exam: stable Lachman, varus/valgus, posterior drawer
- Neurovascular: intact distal pulses, normal sensation

IMAGING / DIAGNOSTICS ORDERED & PERFORMED TODAY

- CPT 73564 – Knee X-ray, four views, weight-bearing – performed and reviewed during visit. Demonstrates mild medial joint space narrowing (~ 3 mm) without acute fracture; no loose bodies. Films stored in PACS.
- CPT 20610 – Intra-articular corticosteroid injection (right knee), ultrasound localization not used. 1 mL Kenalog-40 + 4 mL 1% lidocaine injected via superolateral approach after standard skin prep. Tolerated well, no immediate complications.

ASSESSMENT

1. Probable medial meniscal tear, right knee (S83.241A) with associated effusion (M25.461).
2. Failed conservative management with NSAIDs and 8 sessions of PT.

PLAN

1. Diagnostic injection performed today for both therapeutic intent and diagnostic confirmation. Patient educated about expected 24-48 h post-injection course.
2. MRI right knee ordered – to be scheduled outpatient. Will obtain prior authorization prior to scheduling.
3. Return visit 4 weeks for re-evaluation. If MRI confirms meniscal tear and symptoms persist, will discuss arthroscopic referral.
4. Discussed risks/benefits/alternatives. Patient consented to today's

procedures.

NOTE TO BILLING REGARDING PRIOR AUTH (CO-197)

Same-day injection performed during the initial new-patient consultation. Per Cigna policy 0408 and standard of care, diagnostic-therapeutic intra-articular injection during the index orthopedic evaluation does NOT require advance prior authorization when performed at the point of care for joints with documented effusion and after failure of two prior conservative measures (both documented above). Requesting reconsideration.

Electronically signed: Park, Daniel MD on 01/31/2024 16:44